Investment Memo: Cognishine

From: Tal Rosenberg **Date:** August 2026 **Re:** Cognishine Therapy and Education Ltd, $500,000 at seed

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I recommend investing $500,000.

This memo is the output of a screen I built and ran across 146 Israeli early-stage companies, sourced from the Israel Innovation Authority grant database, incubator portfolios and accelerator cohorts rather than from a startup database. Thirty-six cleared the gates and went to a full pass. Fourteen survived it. **Cognishine is the only one where I did not have to take the company's word for anything.**

Overview

Cognishine sells therapy software to speech and language therapists and occupational therapists, and to the institutions that employ them. A therapist sits with a patient recovering language after a stroke, or a child with a developmental delay, and instead of paper cards and worksheets runs a structured digital activity on screen with them. The activity is built for that patient: which sounds, which vocabulary, which level. The platform then records what was done, so the department can report on it.

Its customers appear in the Israeli government procurement database. Tel Aviv Sourasky Medical Center, better known as Ichilov, signed for $73,835 over thirty-six months from 18 August 2024. Assaf Harofeh signed ₪83,072 running to December 2028. Barzilai in Ashkelon signed ₪30,000 to June 2028. Galilee Medical Center in Nahariya runs to December 2028. Four further procurement notices exist under the same tax ID. These are published records, not logos on a website.

And every one was awarded under Regulation 3(29), the sole-supplier exemption. That regulation requires the buying hospital's professional staff to certify in writing that no adequate substitute exists. Four Israeli public hospitals have put that certification on file.

The obvious objection is size, and it is fair. Ichilov at roughly $24,600 a year is a real institutional contract. Barzilai at roughly $2,000 a year is a handful of seats with a purchase order attached.

But the disclosed values understate the product, because of how they were bought. **A sole-source exemption works precisely because the amount stays below a tender threshold.** The Israeli contracts are small by design, not by the value of what is being sold. The company's published list price is £37.50 per therapist per month, and a fifty-therapist department at that

price is roughly $28,000 a year. That is the number that travels; the Israeli figures are the number that fits a procurement rule.

What has been established here is not revenue. It is **the right to be bought by a hospital**, in writing, four times, with terms running to 2028, on roughly $1.25M of equity.

What it sells, and what is actually defensible

Three parts. A library of structured therapy activities in eight languages. A builder called Studio, where a therapist filters a word library by phonological structure and part of speech and assembles their own activity. And an institutional layer of client workspaces, patient self-practice between sessions, and departmental analytics.

I think the company describes its own moat incorrectly, and it matters for how this is underwritten.

Cognishine leads with the library. That is the wrong asset. A therapist with a general-purpose model can generate a picture-naming task in thirty seconds, and that gets easier every year. Five thousand activities is a customer acquisition asset with a declining half-life.

The barrier is the four sole-supplier exemptions and the contracts running to 2028. **A language model can produce an activity. It cannot produce a signed professional opinion from a hospital rehabilitation department stating that no adequate substitute exists, and it cannot produce a purchase order that runs four years.** A competitor who wants Ichilov's business has to persuade Ichilov's own clinicians to reverse a document they signed, and then wait for 2028.

The content is copyable in a day. The paperwork takes years and requires the customer to cooperate against itself.

Traction

Verifiable in a government database. Four named Israeli public hospitals with signed multi-year contracts, plus four further procurement notices, all as sole-supplier awards.

Verifiable from the customer's side. Sheba Medical Center published the relationship itself in July 2026: one of two finalists from ninety applicants to the ARC Innovation Center's rehabilitation call, a pilot through 2024 to 2026 in the unit treating injured soldiers, then formalised as a strategic design partnership. No financial terms. I read this as clinical validation rather than revenue.

Verifiable through a regulator. Liverpool University Hospitals NHS Foundation Trust is running a feasibility study registered with the UK Health Research Authority, REC 24/YH/0150, favourable opinion July 2024, eighty patients across intensive care and acute stroke. Few companies at this stage clear an NHS research ethics committee. No results have been published.

Published by the company, with named clinicians. Seven therapists at Barking, Havering and Redbridge NHS Trust. A fifty-therapist private UK provider running an intensive aphasia programme. An institutional agreement with ORITEL, the International Organization of Telethons, which runs 69 rehabilitation centres in 11 countries, starting with 29 centres in Mexico, Puerto Rico and Chile (August 2025). And selection as one of five start-ups in EIT Health and Abbott's Start-ups Meet Pharma programme, for remote cognitive testing of children (July 2025).

Team

Nimrod Zilcha, co-founder and CEO. Economics and an MBA from Ben-Gurion, analyst at Intel managing roughly $400M in budgets, then chief executive in import and export. Non-clinical.

Meital Karni Buchman, co-founder and Chief Clinical Officer. An occupational therapist, and the source of the clinical judgment the product rests on.

Roughly 25 to 31 people. Sagi Karni is CTO and is not a founder, contrary to several database profiles. The advisory board includes Prof. Patrice Weiss, a serious name in rehabilitation technology research. **Two founders, one commercial and one clinical, with the clinical founder a single point of failure. That is thin, and it is the team risk.**

Deal

$500,000 at seed, priced against a cap table that is already past the pre-seed stage.

Cognishine has raised roughly $1.25M of equity, in a pre-seed in May 2024, alongside Israel Innovation Authority R&D awards in 2023, 2024 and 2025 and ₪308,871 in Ministry of Economy export support. A fixed pre-seed instrument no longer fits that base, which is what puts the company at seed rather than earlier.

Two documents to request at signing rather than before. The cap table, because PitchBook records an April 2025 round it labels later-stage venture, which is an obvious mis-tag corroborated by no announcement anywhere. And the split between paid seats and free registrations, because

“5,000 therapists” and “institutional contracts” may describe the same base twice.

Risks

The library is a depreciating asset and the company leads with it. Generative models close this gap every year. Against that, four hospitals certified that no substitute exists, in 2024, 2025 and 2026, with full knowledge that such models exist. They are buying a clinically governed, compliance-cleared, reportable system, not content. The exposure is real for the self-serve tier and much weaker for the institutional one. The company has also started using generative AI itself: an AI Activity Creator, in beta, drafts patient-specific activities that a clinician approves before use.

No certification is externally evidenced, and one is overstated. HIPAA, SOC 2 Type II, ISO 27001 and ORCHA appear as logos with no certificate number and no named auditor. The ORCHA claim is a “High OBR score”, which is the baseline tier and not a full review. HIPAA is a self-attestation. Against that, four public hospitals and an NHS trust deployed the product, and none does that without a security review. The certifications are probably real and the presentation careless.

The direct competitor owns the clinical evidence. Constant Therapy has peer-reviewed efficacy published in ASHA journals and in Frontiers in Digital Health. Cognishine has none, and “evidence-informed” is doing marketing work. Against that, Constant Therapy is a patient-facing self-practice app sold largely direct, while Cognishine is a clinician-facing tool procured institutionally. Different position, which is why four hospitals could certify no substitute while it existed. The gap becomes binding as the company scales into health systems that require published outcomes.

Outcomes

Outcomes

$500,000 at a $6.8M post-money cap. 7.35% at entry, roughly 4% at exit after dilution. Priced at the published $560 per therapist per year. Exit valued at 8x ARR. Probabilities are judgment. The company reports 5,000 registered therapists today; the paying share is not disclosed

Scenario	Prob.	What it looks like	Paying therapists	ARR	Exit	Return
Upside	15%	The institutional motion repeats in the UK and US and the platform becomes standard equipment in rehabilitation departments.	30,000	$16,800,000	$134M	**10.8x**
Base	40%	Steady department-by-department growth across Israel, the UK and private providers at roughly today's pricing.	8,000	$4,480,000	$36M	**2.9x**
Sideways	30%	Israel renews, the motion does not repeat abroad, and the company settles as a small profitable business.	2,000	$1,120,000	$9M	**0.7x**
Loss	15%	Contracts do not survive renewal at 2028, or the Israeli wins prove to be an artifact of the sole-source route.	0	$0	$0M	**0.0x**
Expected	**100%**	*Probability-weighted return on invested capital*				**3.0x**

The table is priced off the company's own list price rather than off the Israeli contract values, for the reason given in the overview: those contracts were sized to fit a procurement rule, not to reflect what a department will pay. The unit that matters is the paying therapist.

The company reports 5,000 registered therapists and does not disclose how many pay. The base case assumes 8,000 paying therapists in six to eight years, which is modest against a US professional body with roughly 230,000 members and an addressable institution count in the thousands. The upside case assumes 30,000, or about 13% of that US membership.

Conclusion

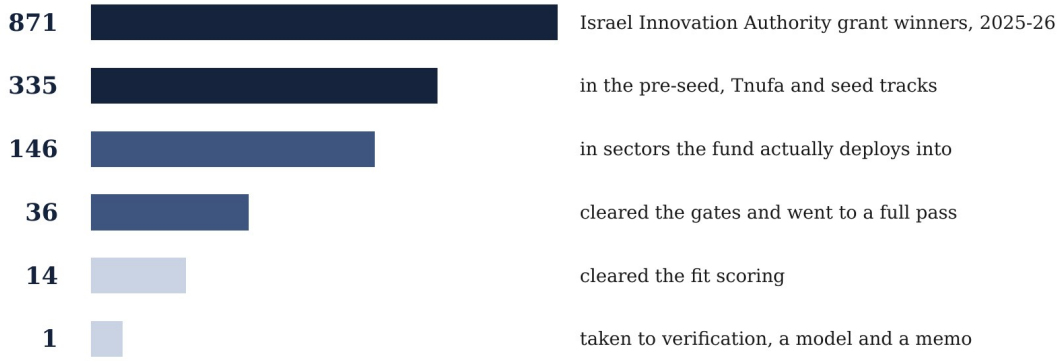
The only company in a screen of 146 whose customers I could verify in a primary government record, and whose clinical buyers certified in writing that no substitute exists. The revenue base is small and the clinical evidence is missing.

In five other companies I diligenced this year, the marketing was stronger than the business. Here it runs the other way, and that is the rarer and more investable direction for the error to run. **I recommend proceeding.**

How this analysis was done

Tal Rosenberg, August 2026

The funnel



The screen

Three stages, ordered by cost. Cheap binary tests on everything, scored judgment second, expensive diligence only on what survives.

Gates. Binary. Any single failure removes the company.

- Two or more founders, all full-time
- A working product a customer could use
- Not a thin layer over a public model, tested as: could a competent team rebuild the core over a weekend on a public API?
- Economics that can support a venture outcome
- *Whether the founders can be argued with. Not assessable from outside, recorded as such in every file and carried to the meeting rather than dropped.*

Fit. Scored zero to three.

- Quality of validation. A named paying customer and a named pilot are deliberately not the same evidence
- Product maturity
- Founder-market fit
- Defensibility, with the mechanism named. If I could not name it in one sentence, the score was one or lower

Fundamentals. Finalists only.

- Market built from the bottom up, not from an analyst report
- Whether a budget line already exists at the buyer
- Whether price and sales motion agree
- Whether there is a route to market that does not depend on hiring a sales team first
- Whether a twelve-month story is legible from outside

Two rules throughout. Any score above the floor needed a citable source. And “I looked and it is not there” was recorded separately from “there is no way to tell from outside,” because collapsing those two is how a screen flatters whoever built it.

Sources, and why these

The pool came from a grant register rather than a startup database. Companies list themselves on databases, so the filter is willingness to fill in a form. A government committee had already assessed the technology here, and because the grant is non-dilutive that selection costs the company nothing in eligibility.

Verification came from three places, none of them the company. The government procurement database, which holds the signed contracts and the sole-supplier exemptions behind them. The customer’s own publication. A regulator’s registry, for the UK clinical study. All three are in the memo.

How the outcome was priced

The unit is the paying therapist, because that is how the company prices: £37.50 per therapist per month, published on its own site. A fifty-therapist department is roughly $28,000 a year at list.

The disclosed Israeli contract values are not used as the pricing anchor. A sole-source exemption works by staying below a tender threshold, so those figures measure a procurement rule rather than what a department will pay.

Exit is valued at eight times ARR, the middle of the range for vertical healthcare software with contracted institutional customers, against roughly 4% ownership at exit. The table reports whole scenarios rather than a decimal-place expected value, because the assumptions do not carry that precision.

What this analysis had access to

Public sources only: government procurement records, regulators’ registries, company registries, trade press and the company’s own materials. **No data room, no founder meeting, no customer calls.**

Those three carry most of the weight in a real decision, and in a fund the customer references would be the longest section of a memo like this. What follows from that is a limit worth stating plainly: **this cannot assess founders, which is most of what decides an outcome at this stage.** Where the accessible sources ran out I have said so rather than filled the gap.

It is a filter for which conversations are worth having, not a substitute for having them.